

HOSPITAL DISCHARGE SUMMARY

PATIENT DEMOGRAPHICS

Name: Patient 20

Age: 65 years old

Sex: Female

MRN: 000-TEST-20

Admission Date: 06/10/2026

Discharge Date: 06/17/2026

Length of Stay: 7 days

PRIMARY DIAGNOSIS

Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage II (moderate airflow limitation)

SECONDARY DIAGNOSES

- Rheumatoid Arthritis (RA), stable on current regimen
- Gastroesophageal Reflux Disease (GERD)
- Hypertension
- Hyperlipidemia

HOSPITAL COURSE

Patient 20 presented to Metro Medical Center via ED with acute dyspnea, productive cough, and increased sputum purulence × 4 days. Initial vitals: HR 108, RR 24, BP 152/88, SpO2 88% on room air. Pulmonary function testing on admission revealed FEV1/FVC ratio of 58% (predicted FEV1 52%), consistent with GOLD Stage II moderate COPD. Chest X-ray demonstrated hyperinflation without acute infiltrate.

Patient was initiated on high-dose nebulized albuterol/ipratropium q2h, IV methylprednisolone 125 mg q6h, and empiric azithromycin for suspected infectious trigger. Overnight observation showed minimal improvement; SpO2 remained 89–91% on supplemental O2 at 2L NC. Arterial blood gas on hospital day 2 revealed pH 7.34, pCO2 48 mmHg—permissive hypercapnia noted but patient clinically improving. By hospital day 4, SpO2 improved to 93–95% on 1L O2 at rest; nebulizer frequency reduced to q4h.

Patient tolerated transition to home inhalers by day 5. Chest X-ray on discharge showed interval improvement in hyperinflation. Rheumatology consulted regarding RA flare concern (elevated ESR 42, CRP 8.2); continued methotrexate without adjustment. GI regimen optimized for GERD management. Patient discharged on hospital day 7 in stable condition, SpO2 94% on room air at rest, with structured pulmonary rehabilitation referral.

DISCHARGE MEDICATIONS

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Daily inhalation | Ongoing |

| Formoterol/Budesonide (LABA/ICS) | 200/6 mcg | 2 puffs BID | Ongoing |

| Albuterol (SABA) | 90 mcg | 2 puffs q4–6h PRN | Ongoing |

| Prednisone (systemic steroid taper) | 40 mg daily × 3 days, then 30 mg daily × 3 days, then 20 mg daily × 3 days, then 10 mg daily × 2 days | Taper schedule | 11 days total |

| Azithromycin | 250 mg | Daily PO | 3 more days (total 5-day course) |

| Omeprazole | 20 mg | Daily PO | Ongoing |

| Methotrexate | 15 mg | Weekly SC injection | Ongoing (RA) |

| Lisinopril | 10 mg | Daily PO | Ongoing |

ACTIVITY & MOBILITY

Patient is fully ambulatory without restrictions. Encourage gradual increase in activity as tolerated